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Melasma

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O que é?

Melasma é uma hiperpigmentação da pele, decorrente da deposição aumentada de melanina, proteína que garante a coloração da pele e evita os danos da radiação ultravioleta no DNA. O transtorno resulta na formação de manchas castanho-escuras ou marrom-acinzentadas, com limites bem demarcados, mas formato irregular. Embora se localizem preferencialmente na face, na região das maçãs do rosto, da testa, do lábio superior, no queixo e nas têmporas, as lesões também podem surgir no colo, pescoço e antebraços. O tamanho das manchas pode variar bastante. Em alguns casos, elas chegam a tomar as duas faces completamente.

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O que é?

Melasma é uma condição crônica e recidivante. Mais frequente nas mulheres em fase reprodutiva, entre 20 e 50 anos, do que nos homens (apenas 10% são afetados), é raro manifestar-se antes da puberdade.

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O que causa?

- Inflamação
- Radiação - Sol
- Anticoncepcionais
- Gestação - Menopausa
- Maquiagem
- Filtro solar e cosméticos
- Ansiedade
- Disfunção da tireoide;
- Drogas para tratamento da hipertensão ou epilepsia.
- Resitência insulínica

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frontiers | Frontiers in Microbiology

TYPE Original Research
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Correlation analysis between gut microbiota characteristics and melasma

Cong Liu[1], Dan He[2], Anye Yu[1], Yaru Deng[1], Li Wang[2]* and Zhiqi Song[1]*

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In recent years, many studies have shown that the gut microbiota can affect the occurrence and development of a variety of human diseases. A variety of skin diseases are related to the regulation of the gut–skin axis, such as psoriasis, atopic dermatitis, and acne. Gut microbial dysbiosis can promote the development of these diseases. The gut microbiota can affect estrogen metabolism, β-glucuronidase secreted by the gut microbiota can promote the reabsorption of estrogen by the gut, and estrogen is transported to other parts of the body through the circulatory system. The occurrence and development of melasma are closely related to abnormal metabolism of estrogen. The relationship between the structure of the gut microbiota and melasma remains unclear. Epidemiological surveys were conducted in patients with melasma and healthy subjects (control group) in this study. The feces were collected for 16S rRNA sequencing analysis of the gut microbiota. To compare the similarities and differences in species diversity of the gut microbiota between these two groups, we calculated the α-diversity and β-diversity indices and analyzed the differences between them. We found that the abundance of *Collinsella* spp., *Actinomyces* spp. (belonging to Actinobacteria), *Parabacteroides* spp., *Bacteroides* spp., *Paraprevotella* spp. (belonging to Bacteroidetes), *Blautia* spp., and *Roseburia* spp. (belonging to Firmicutes) in the melasma group were significantly different compared with that in the healthy group. The largest difference was found in Actinobacteria ($p<0.05$), and there were also significant differences in the abundance of Coriobacteriia, Actinobacteria, Coriobacteriales, Coriobacteriaceae, and *Collinsella* spp. between the two groups (all $p<0.05$). Many of these differences in the microbiota were closely related to the production of β-glucuronidase and the regulation of estrogen synthesis or metabolism. Changes in the gut microbiota structure and the biological effects of *Collinsella* spp. in the microbiota in patients with melasma can play an important role in the occurrence and development of melasma by affecting the body's estrogen metabolism. This study provides a theoretical basis and experimental data reference for future studies on the relationship between the gut microbiota and melasma, and may be helpful for the prevention and treatment of melasma.

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J Cosmet Dermatol. 2021 Nov;20(11):3656-3660. doi: 10.1111/jocd.14014. Epub 2021 Mar 2.

Non-alcoholic fatty liver and lipid profile status in patients with melasma: A case-control study

Mohammadreza Ghassemi [1], Saba Hosseinchi [2], Farnoosh Seirafianpour [3], Milad Dodangeh [3], Azadeh Goodarzi [1]

Affiliations + expand

PMID: 33609335 DOI: 10.1111/jocd.14014

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Abstract

Background: There are pieces of evidence regarding the association between melasma and metabolic syndrome. To assess the prevalence and grade of nonalcoholic fatty liver and lipid profile status in patients with melasma, we designed this case-control study.

Methods: This study was performed on 30 consecutive women with melasma who referred to the dermatology clinic of the hospital as the case group and 34 healthy women. For all participants, serum lipid profiles and liver enzymes were checked. Also, the existence of fatty liver was assessed by ultrasonography.

Results: Except for serum level of LDL (low-density lipoprotein) that was significantly higher in the melasma group than in the healthy group (104.23 ± 25.00 mg/dl versus 89.85 ± 23.00 mg/dl, p = 0.020), the level of other parameters including other lipid profiles, blood sugar, or liver enzymes was similar in both groups. In ultrasonography, the overall prevalence of fatty liver was 23.3% in the melasma group and 20.6% in the control group, and no difference was found between the two groups in grade of fatty liver (p = 0.791).

Conclusion: The study showed a higher serum LDL level in patients with melasma compared to women without melasma, but there was no difference between the groups in prevalence or grade of fatty liver.

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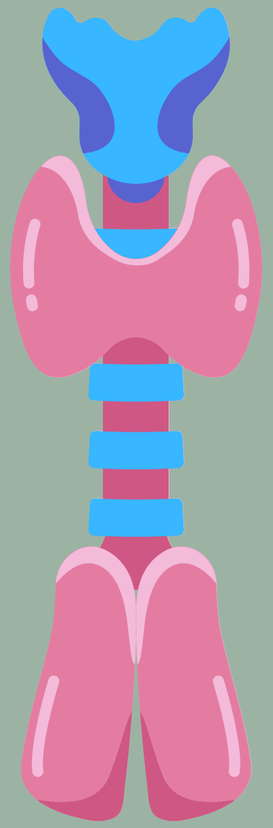

Meta-Analysis > Int J Dermatol. 2019 Nov;58(11):1231-1238. doi: 10.1111/ijd.14497.
Epub 2019 May 31.

Melasma and thyroid disorders: a systematic review and meta-analysis

Motahhareh Kheradmand [1], Mahdi Afshari [2], Giovanni Damiani [3 4 5], Saeid Abediankenari [6 7], Mahmood Moosazadeh [1]

Abstract

Background: Thyroid hormones may play a key role in melasma; however, melasma link with thyroid disorders remains controversial.

Objectives: To compare the serum levels of thyroid-stimulating hormone (TSH), T4, T3, anti-thyroid peroxidase (anti-TPO), and antithyroglobulin between patients with melasma and control group using meta-analysis.

Methods: We screened 10 databanks and search engines, searched mesh and nonmesh terms. The identified evidences were reviewed and quality assessed using the Newcastle-Ottawa Scale (NOS). The heterogeneity between the primary results was investigated using Cochrane and I-square indices. Random effect model was applied to combine the standardized mean differences of thyroid function indicators between patients with and without melasma. P values meta-analysis was used to investigate the association between anti-TPO and melasma.

Results: We included seven studies, 473 cases, and 379 controls that had been investigated. The total standardized mean differences (95% confidence intervals) of TSH, T3, T4, and antithyroglobulin antibody between cases and controls were estimated to be 0.33 (0.18, 0.47), -0.01 (-0.20, 0.19), -1.50 (-2.96, -0.04), and 0.62 (0.14, 1.11), respectively. The corresponding figures among women were 0.35 (0.17, 0.52), 0.10 (-0.17, 0.38), -2.75 (-6.30, 0.81), and 0.99 (0.14, 1.83), respectively. P value of meta-analysis showed a significant relationship between anti-TPO serum level and melasma (Fisher = 26.80, P = 0.020).

Conclusion: Serum levels of TSH, anti-TPO, and antithyroglobulin antibody were significantly higher in patients with melasma than those without melasma. Moreover, these differences were more severe among women with melasma.

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PRIMEIRO PASSO

Protocolo Desinflama Conecta

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Review > Cutis. 2019 Mar;103(3):153-156.

The role of diet in preventing photoaging and treating common skin conditions

Yssra S Soliman [1], Peter W Hashim [2], Aaron S Farberg [2], Gary Goldenberg [3]

Affiliations + expand

PMID: 31039233

Abstract

As interest in complementary and alternative medicine has grown, the relationship between diet and skin health has become an active area of research. Various supplements, plant derivatives, and antioxidants have gained attention as possible tools to prevent signs of aging and improve skin conditions. As such, knowledge of clinical trial data is important to counsel patients appropriately on risks and benefits of these complementary treatments and lifestyle modifications. Herein, we review the role of diet and supplements in preventing photoaging and treating common skin conditions.

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Prevenção

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Suplementos

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CSC

- Probióticos - Lact. Jonhsonii
- Ômega 3
- Vit. C, A, E (diminui o grau de pigmentação)
- Carotenóides e tocoferol, flavonóides , protocianidinas, licopeno
- Vitis vinifera, cacau, vinho, uva, ext. semente de uva, transresveratrol
- Ext seco de romã 90 % ácido elágico - clarear

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CSC

- Picnogenol (pinus pinaster) - protege da radiação
- Ext. óleo da azeitona (5% de hidroxitirosol)
- Polypodium leucotomos
- Colágeno hidrolisado
- Melatonina - sono !
- Metformina tópica - Berberina
- Fotoprotetores

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Review > J Drugs Dermatol. 2020 Aug 1;19(8):788-792. doi: 10.36849/JDD.2020.5079.

The Therapeutic Use of Antioxidants for Melasma

Kayla M Babbush, Remy A Babbush, Amor Khachemoune

PMID: 32845595 DOI: 10.36849/JDD.2020.5079

Abstract

Melasma is a chronic dermatologic condition with an incompletely understood pathogenesis and well-demonstrated impact on patient quality of life. Melasma is a common cause for seeking dermatologic care, and with no universally efficacious therapy or cure, com-bination treatment is the best approach for many cases. Numerous studies have demonstrated the role of oxidative stress in patients with melasma, prompting investigation into several antioxidants for melasma therapy. In this review, we discuss the well-defined role of oxidative stress in melasma and the therapeutic efficacy of various antioxidants for patients suffering from melasma. We focus our discussion on studies investigating the role of vitamin C, azelaic acid, cysteamine, glutathione, carotenoids, and numerous other antioxidants in disorders of hyperpigmentation. There is promising evidence for the use of these antioxidants, as topical, oral, and intra-venous preparations, both in isolation and in conjunction with other melasma therapies. J Drugs Dermatol. 2020;19(8):788-792. doi:10.36849/JDD.2020.5079.

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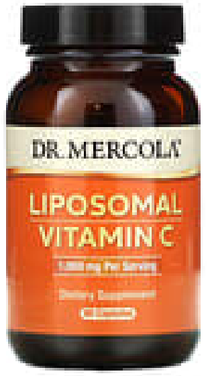
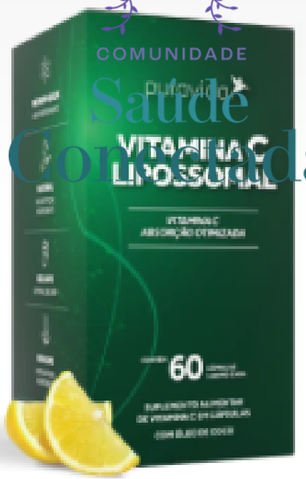
Solgar, Ester-C Plus, vitamina C, 500 mg, 100 Cápsulas Vegetais



Dr. Mercola, Vitamina C Lipossomal, 500 mg, 60 Cápsulas

Burt's Bees, Crianças, Gomas de Suporte Imunológico, Laranja, 50 Gomas



VITAMINA C LIPOSSOMAL 1100 MG

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Country Life, Complexo Carotenoide, 60 cápsulas softgel

Jarrow Formulas, CarotenoTudo, Complexo de Carotenoides Misto, 60 Cápsulas Softgel

Nature's Way, Luteína, 20 mg, 60 Cápsulas Gelatinosas

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Red Orange Complex

A sinergia das laranjas vermelhas a favor do tempo

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DOSAGEM USUAL 100mg ao dia		Uso in

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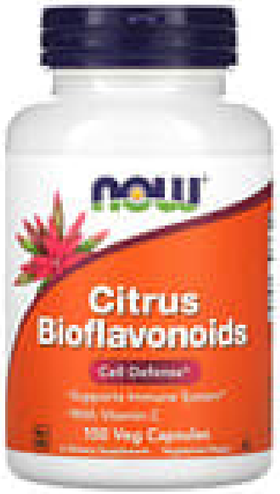

Now Foods, Bioflavonoides Cítricos, 100 Cápsulas Vegetais

Doctor's Best, Tocotrienóis com EVNol SupraBio, 50 mg, 60 Cápsulas Softgel

Life Extension, Tocotrienóis Superabsorvíveis, 60 Cápsulas Softgel

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Vitis Vinifera - Farinha de Semente de Uva Orgânica

Plantopia
60 g

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21st Century, Extrato de Vinho Tinto com Resveratrol, 90 Cápsulas

Nature's Way, Mistura Premium, Resveratrol, 60 Cápsulas Veganas

Now Foods, Potência Extra, Semente de Uva, 250 mg, 90 Cápsulas Vegetais

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ife Extension, Extrato de Romã, 30
Cápsulas Vegetarianas

Novo

Swanson, Extrato de Romã, 250
mg, 60 Cápsulas

Source Naturals, Extrato de Romã,
500 mg, 60 Comprimidos

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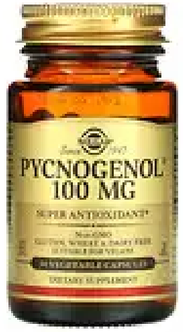
Swanson, Pycnogenol, 100 mg, 30 cápsulas



Solgar, Pycnogenol, 100 mg, 30 Cápsulas Vegetais

Source Naturals, Pycnogenol, 100 mg, 60 comprimidos

Now Foods, Pycnogenol, 100 mg, 60 Cápsulas Vegetais

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Now Foods, Extrato de Folha de Azeitona, 100 Cápsulas Vegetais

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Nature's Way, Folha de Oliveira, 500 mg, 100 Cápsulas Veganas

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OLI-OLA

A excelência do fruto da oliveira

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USO ISOLADO	USO ASSOCIADO	Uso in
300mg ao dia	150 a 300mg ao dia	

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Informações Nutricionais

Dose: 2 Cápsulas Vegetarianas

Doses por Recipiente: 60

	Quantidade por Dose	% Valores Diários
Vitamina C (como ácido ascórbico da Red Orange Complex®)	5,5 mg	6%
Ácido Nicotínico (como Nicotinamida)	500 mg•	3.125%
Extrato de Polypodium leucotomos (folha)	240 mg	**
Red Orange Complex® [Laranjas vermelhas da Sicília (Citrus sinensis var. Extrato de Moro, Sanguinello, Tarocco) (fruta e casca)]	100 mg	**

** Valores Diários não estabelecidos. • NE (equivalentes de niacina).

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> Int J Dermatol. 2020 May;59(5):572-575. doi: 10.1111/ijd.14827. Epub 2020 Mar 5.

Melatonin and oxidative stress in melasma – an unexplored territory; a prospective study

Rashmi Sarkar [1], Shanthy Devadasan [1], Vikrant Choubey [1], Binita Goswami [2]

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Como usar ?

1. Escolher DOIS ou TRÊS suplementos para MELASMA
2. Pode acrescentar esses dois aos escolhidos de outras imersão
3. Usar na dose indicada do fabricante
4. Utilizar por até 3 meses e rotacionar
5. Pode tomar com chá de folhas de amora ou verde

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COLLAGEN PROTEIN SACHE

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Clinical Trial > Phytother Res. 2002 Sep;16(6):567-71. doi: 10.1002/ptr.1085.

Treatment of melasma with Pycnogenol

Z Ni [1], Y Mu, O Gulati

Affiliations + expand

PMID: 12237816 DOI: 10.1002/ptr.1085

Abstract

Melasma (or chloasma) is a common disorder of cutaneous hyperpigmentation predominantly affecting sun-exposed areas in women. The pathogenesis of melasma is not fully understood and treatments are frequently disappointing and often associated with side effects. Pycnogenol is a standardized extract of the bark of the French maritime pine (Pinus pinaster), a well-known, potent antioxidant. Studies in vitro show that Pycnogenol is several times more powerful than vitamin E and vitamin C. In addition, it recycles vitamin C, regenerates vitamin E and increases the endogenous antioxidant enzyme system. Pycnogenol protects against ultraviolet (UV) radiation. Therefore its efficacy in the treatment of melasma was investigated. Thirty women with melasma completed a 30-day clinical trial in which they took one 25 mg tablet of Pycnogenol with meals three times daily, i.e. 75 mg Pycnogenol per day. These patients were evaluated clinically by parameters such as the melasma area index, pigmentary intensity index and by routine blood and urine tests. After a 30-day treatment, the average melasma area of the patients decreased by 25.86 +/- 20.39 mm(2) ($p < 0.001$) and the average pigmentary intensity decreased by 0.47 +/- 0.51 unit ($p < 0.001$). The general effective rate was 80%. No side effect was observed. The results of the blood and urine test parameters at baseline and at day 30 were within the normal range. Moreover, several other associated symptoms such as fatigue, constipation, pains in the body and anxiety were also improved. To conclude, Pycnogenol was shown to be therapeutically effective and safe in patients suffering from melasma.

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Review > J Cosmet Dermatol. 2014 Dec;13(4):324-8. doi: 10.1111/jocd.12119.

A review of nicotinamide: treatment of skin diseases and potential side effects

Heidi M Rolfe [1]

Affiliations + expand

PMID: 25399625 DOI: 10.1111/jocd.12119

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Abstract

Nicotinamide, also known as niacinamide, is the amide form of vitamin B3. It is a precursor of essential coenzymes for numerous reactions in the body including adenosine triphosphate (ATP) production. Nicotinic acid, also known as niacin, is converted into nicotinamide in the body. The use of topical nicotinamide in the treatment of acne vulgaris; melasma; atopic dermatitis; rosacea; and oral nicotinamide in preventing nonmelanoma skin cancer is discussed. The possible side effects and consequences of excessive nicotinamide exposure are reviewed, including suggestions nicotinamide might have a role in the development of diabetes, Parkinson's disease, and liver damage.

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Review > J Dermatolog Treat. 2022 Jun;33(4):1816-1837.
doi: 10.1080/09546634.2021.1914313. Epub 2022 Mar 23.

Melasma treatment: a systematic review

Nicoleta Neagu [1], Claudio Conforti [2], Marina Agozzino [2], Giovanni Francesco Marangi [3], Silviu Horia Morariu [1], Giovanni Pellacani [4], Paolo Persichetti [3], Domenico Piccolo [5], Francesco Segreto [3], Iris Zalaudek [2], Caterina Dianzani [3]

Affiliations + expand

PMID: 33849384 DOI: 10.1080/09546634.2021.1914313

Abstract

Melasma is a common chronic refractory disorder of pigmentation affecting people with darker skin types. Overall prevalence varies between 8.8% and 40%, depending on the ethnicity of the population and the geographical area. Therapeutic management of melasma is challenging, with high recurrence rates which significant impacts on the quality of life. No single treatment is universally efficacious. Systemic treatments with tranexamic acid and polypodium leucotmatous had promising results, although the former was related to systemic side effects. Microneedling and peeling were also efficacious, although their superiority to topical hydroquinone, the gold standard in melasma treatment, remains to be established. Similarly, laser and light devices have been beneficial. However, recurrence rates remain high in all treatment groups. Combination therapies, either in double or triple combinations yielded the best results when compared to single terapies. Treatment choice should be made after Wood's lamp examination, as well as dermatoscopic evaluation, in order to select the best treatment option, targeted at each melasma subtype.

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Dermatologista

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Protetor solar físico

#1 Photoderm Nude Touch Bioderma
#2 Photoage Mineral Mousse FPS50 Dermage
#3 Physical Mate UV Defense FPS50 SkinCeuticals
#4 Protetor Solar Mineral Quintal Mineralis FPS50
#5 Protetor solar físico da KHOR
#6 Sunprise Mild Airy Finish, Etude
#7 Photoage Mineral Fluido
#8 Pink Stick FPS 90
#9 Protetor Solar Mousse Adcos
#10 Bioré UV Perfect Milk

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Randomized Controlled Trial > J Cosmet Dermatol. 2020 May;19(5):1161-1164.
doi: 10.1111/jocd.13145. Epub 2019 Sep 10.

Topical metformin in the treatment of melasma: A preliminary clinical trial

Ravikumar Banavase Channakeshavaiah [1], Naveen Kumar Andanooru Chandrappa [1]

Review > Int J Mol Sci. 2020 Nov 25;21(23):8960. doi: 10.3390/ijms21238960.

A Molecular Perspective on the Potential Benefits of Metformin for the Treatment of Inflammatory Skin Disorders

Ji-Eun Chang [1], Min Sik Choi [2]

Review > Aten Primaria. 2022 Jun;54(6):102354. doi: 10.1016/j.aprim.2022.102354.
Epub 2022 May 12.

[The role of metformin in the treatment of dermatological diseases: A narrative review]

[Article in Spanish]

Juan Monte-Serrano [1], Pablo Villagrasa-Boli [2], Joana Cruañes-Monferrer [3], Patricia Arbués-Espinosa [4], Sara Martínez-Cisneros [2], Miguel Fernando García-Gil [5]

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Randomized Controlled Trial > J Cosmet Dermatol. 2022 Jun;21(6):2508-2515.
doi: 10.1111/jocd.14953. Epub 2022 Apr 9.

Safety and efficacy of topical metformin 30% cream versus triple combination cream (Kligman's formula) in treating melasma: A randomized controlled study

Eman Said AboAlsoud [1], Ramadan Mohamed Eldahshan [1], Hassan AbouKhodair Mohammed [1], Mohamed L Elsaie [2]

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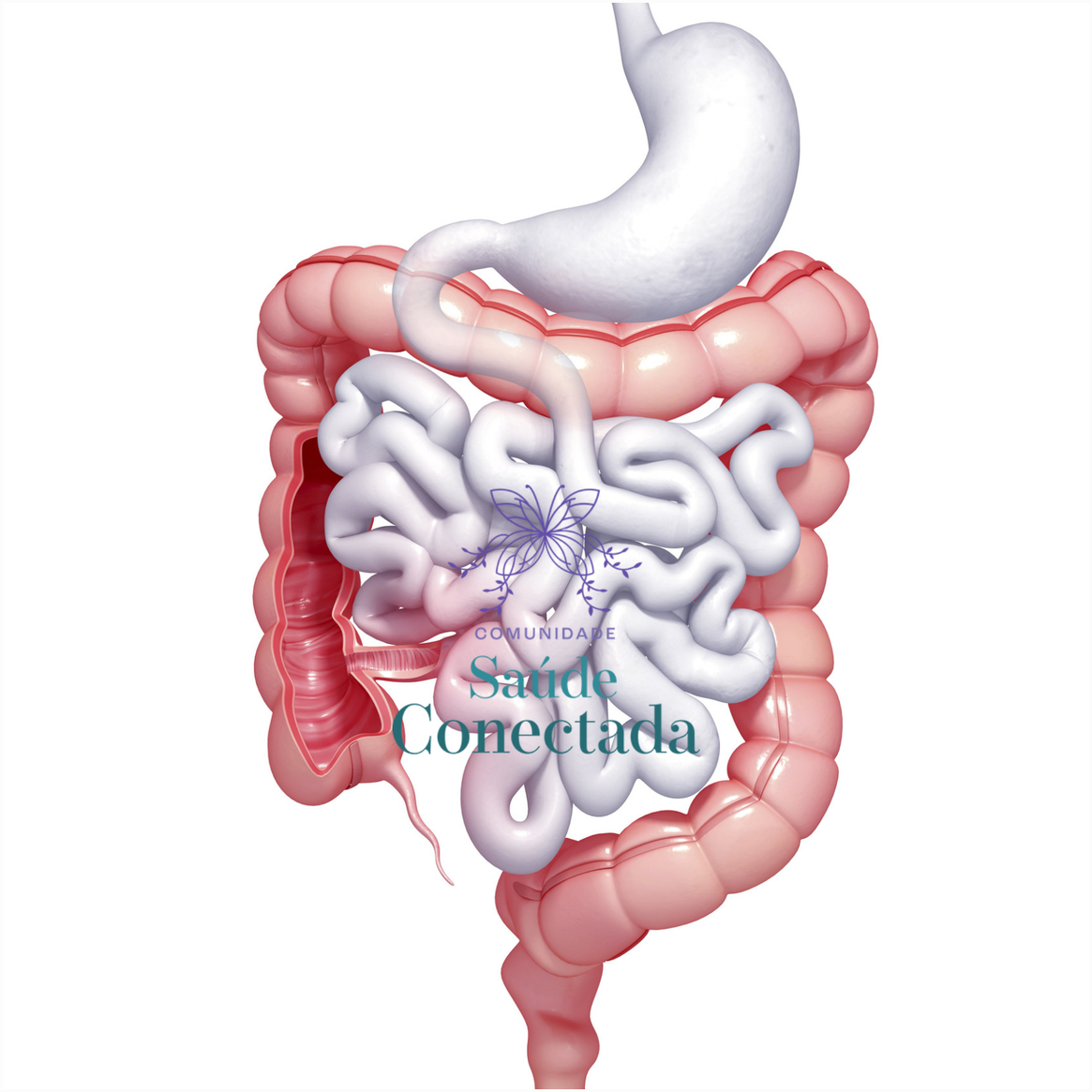

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Dermato-Endocrinology 1:5, 275-279; September/October, 2009; © 2009 Landes Bioscience

Probiotics for photoprotection

Audrey Guéniche,[1,*] David Philippe,[2] Philippe Bastien,[1] Stephanie Blum,[2] Elif Buyukpamukcu[2] and Isabelle Castiel-Higounenc[1]

[1]L'Oréal Recherche; Centre Charles Zviak; Clichy, France; [2]Nestle Research Center; Lausanne, Switzerland

Key words: *Lactobacillus johnsonii*, skin, immune homeostasis, stress, food supplement, probiotics, photoprotection

Specific strains of probiotic, have been identified as beneficial to influence the composition and/or metabolic activity of the endogenous microbiota and some of these strains have been also shown to inhibit the growth of a wide range of enteropathogens. The first aim of using probiotics has been to improve the composition of the intestinal microbiota from a potentially harmful composition towards a composition that would be beneficial to the host.

Beyond their capacity to influence positively the composition of the intestinal microbiota, several lines of evidence suggest that some probiotic bacteria can modulate the immune system both at the local and systemic levels thereby improving immune defense mechanisms and/or downregulate immune disorders such as allergies or intestinal inflammation.

Skin reflects the general health status and aging. Different human trials widely suggest that probiotic supplementation might be useful in the management of atopic dermatitis. Based on these properties it appears that, beyond the gut, probiotics might exert their benefits at the skin level.

In a randomized double blind placebo-controlled clinical trial, we investigated whether the probiotic bacteria *Lactobacillus johnsonii* NCC 533 (La1) could modulate the cutaneous immune homeostasis altered by solar-simulated UV exposure in humans. After, UV exposure to twice 1.5 MED, we demonstrated that La1 intake facilitated an earlier recovery of Epidermal cells allostimulatory function. Thus, this clinical data strengthen the assumption that certain probiotics can contribute to modulate skin immune system leading to the preservation of the skin homeostasis. Altogether the data affords the possibility of designing new strategies based on a nutritional approach for the prevention of UV-induced damaging effects.

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Fórmula para melasma

Oli Ola 300 mg
Picnogenol 75 mg
Polypodium leocotomos 250 mg
Vitamina C 100 mg

Tomar 1 caps ao dia

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CSC

Mascaras naturais

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Máscara de cúrcuma e água de arroz:

Coloque por uma noite de molho um arroz nutritivo (como o arroz vermelho). Pela manhã, coe e retire a água. Pegue de duas a três colheres da água do arroz e adicione cúrcuma em pó até formar uma pasta, acrescente uma colher de chá de mel e aplique no rosto, deite-se e permaneça por 20 minutos (tempo mínimo). Cuidado ao aplicar esta máscara: se a cúrcuma cair em tecidos ou em sofás ela mancha.

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Mamão e mel

Adicione 2 colheres de mel em uma xícara pequena de purê de mamão. Aplique a mistura no rosto e lave-a após meia hora. Repita isso uma vez por semana.

IMPORTANTE: todas as vezes que lavar o rosto utilize água morna ou fria. A água quente, o calor aumentam o melasma.

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Aplique gel de aloe vera fresco na área afetada. Massageie suavemente por alguns minutos e lave com água morna. Faça isso duas vezes ao dia até ver uma diferença visível.

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Dilua uma colher de suco de limão em água e aplique na área afetada. Deixe por meia hora e depois lave.

Faça isso duas vezes ao dia por pelo menos 3-4 semanas.

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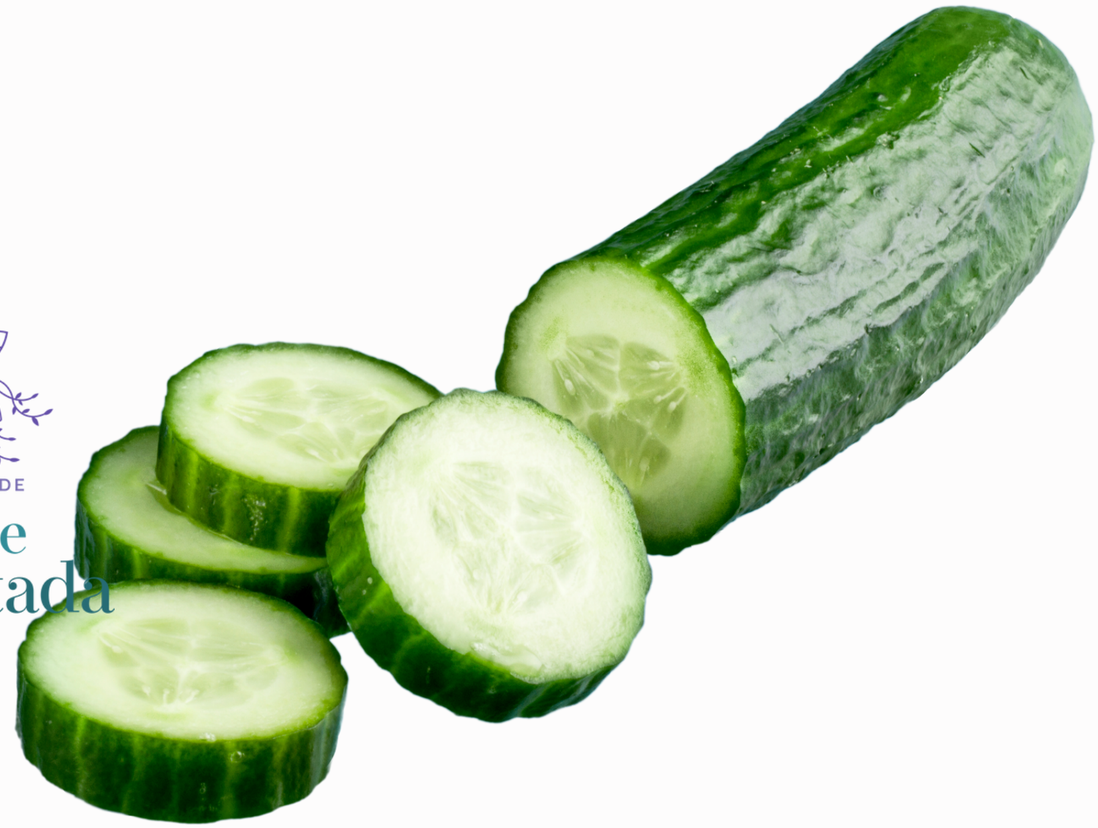
Esprema o suco do pepino ralado e aplique no rosto. Deixe por meia hora antes de enxaguar. Repita uma vez por semana durante 2-3 meses.



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Misture o suco de cebola branca e o vinagre de maçã em quantidades iguais. Aplique esta mistura na área afetada. Lave-o depois de meia hora. Você pode repetir isso duas vezes por dia para obter melhores resultados.

Faça um teste de adesivo no cotovelo antes de aplicar o suco de cebola e vinagre de maçã na pele. Você pode continuar a aplicá-lo no rosto se não tiver irritação na pele ou sensação de queimação por 24 a 48 horas após a aplicação.

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A água do chá preto/ verde pode ser usada como um tratamento de clareamento. As propriedades adstringentes do chá acalmam e acalmam a pigmentação inflamatória e também é muito hidratante por natureza. Use uma bola de algodão para aplicar chá preto embebido em manchas escuras de melasma em seu rosto. Faça isso duas vezes por dia para obter os melhores resultados.

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Tome 1 colher de chá de pó de sândalo misture com 1 colher de chá de suco de laranja para fazer uma pasta grossa aplique-o sobre as manchas escuras e áreas afetadas.

Deixe agir por 20 minutos para secar enxágue com água morna e seque com uma toalha. Repita isso duas ou três vezes por semana

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- Máscara com feno grego em pó
- Leite orgânico com aveia
- Argila branca
- Óleo de copaiba 1 a 3 gotas na mao, espalhar e aplicar
- Óleo de ricino
- Argila com carvão ativado

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Máscara de

INGREDIENTES

- 1 colher de chá de argila verde (para peles muito oleosas) ou branca (para peles sensíveis)
- 1/4 colher de chá de carvão ativado
- 1 colher de chá de vinagre de maçã de boa qualidade, água ou água termal

MODO DE PREPARO

- Lave o rosto e aplique a máscara no rosto seco, evitando o contorno dos olhos.
- Espere secar completamente (em torno de 10 minutos) e retire com água morna.
- Hidrate como de costume!

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Alimentos ricos em antioxidantes

Berries (mirtilos, cranberries, amoras, framboesas, romã, fruta do dragão, kiwis, etc)
Feijão (feijão, feijão preto, feijão carioca, cacau, etc)
Legumes (tomate, cebola, alcachofra, brócolis, repolho roxo, beterraba, etc)
Vegetais de folhas verdes e ervas (couve, espinafre, acelga, alcachofra, coentro, tomilho, manjericão, etc)
Especiarias (açafrão, canela, cravo, cominho, gengibre, etc)
Nozes (pecãs, castanhas do Pará, nozes, avelãs, etc.)

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Alimentos alergênicos comuns:

Ovos
Leite e produtos lácteos
Soja e derivados de soja
Glúten de trigo
Peixe
Marisco
nozes
amendoim

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Exemplo de suplementação

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COLLAGEN PROTEIN SACHE

Onde comprar?
Site das marcas
Mercado Livre (ver pontuação do vendedor)
Amazon
Iherb

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